Investment Memo: Cognishine

From: Tal Rosenberg **Date:** August 2026, revised October 2026 **Re:** Cognishine Therapy and Education Ltd, $500,000 at seed

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I recommend a first meeting, and a $500,000 seed investment if three things check out: paying seats and revenue, the cap table, and two compliance steps for the NHS and US health systems.

This memo is the output of a screen I built and ran across 146 Israeli early-stage companies, sourced from the Israel Innovation Authority grant database, incubator portfolios and accelerator cohorts rather than from a startup database. Thirty-six cleared the gates and went to a full pass. Fourteen survived it. **Cognishine is the only one whose customers I could find in a primary government record.**

Overview

Cognishine sells therapy software to speech and language therapists and occupational therapists, and to the institutions that employ them. A therapist sits with a patient recovering language after a stroke, or a child with a developmental delay, and instead of paper cards and worksheets runs a structured digital activity on screen with them. The activity is built for that patient: which sounds, which vocabulary, which level. The platform then records what was done, so the department can report on it.

Six Israeli public hospitals have published notices of intent to buy it in the government procurement register. Tel Aviv Sourasky Medical Center, better known as Ichilov, approved up to $73,835 including VAT over thirty-six months from 18 August 2024. Shamir, better known as Assaf Harofeh, ₪83,072 to December 2028. Tzafon in Poriya, ₪54,000 to September 2027. Barzilai in Ashkelon, ₪30,000 to June 2028. Galilee Medical Center in Nahariya runs to December 2028, with no amount published. The Shoham geriatric centre, ₪60,000 for one year, ended in April 2026. These are public records, not logos on a website, though a notice sets a ceiling and is not proof of payment.

Every notice rests on Regulation 3(29), the sole-supplier exemption. It requires the ministry’s professional authority to state in writing that the supplier is the only one able to deliver. The notice is then published for at least ten working days so that competitors can object, and a tender committee has to approve it. Six hospitals have taken Cognishine through that process.

The obvious objection is size, and it is fair. Across the active notices the approved ceilings come to about ₪150,000 a year. Ichilov, at roughly

$24,600 a year, is a real institutional purchase. Barzilai, at roughly $2,000 a year, is a handful of seats with a purchase order attached.

The contracts are small because each hospital licenses a handful of therapists, not because a procurement rule caps them. The sole-supplier exemption has no ceiling on value. **The number that matters is how far a hospital expands once it has bought.** The company's list price is £37.50 per therapist per month on its Pro plan, so a fifty-therapist department would pay roughly $28,000 a year. No Israeli hospital is buying at that scale yet.

What has been established here is not revenue at scale. It is **the right to be bought by a public hospital**, in writing, six times, with terms running to 2028.

What it sells, and what is actually defensible

Three parts. A library of structured therapy activities in eight languages. A builder called Studio, where a therapist filters a word library by phonological structure and part of speech and assembles their own activity. And an institutional layer of client workspaces, patient self-practice between sessions, and reporting at organisation, therapist and client level.

I think the company describes its own moat incorrectly, and it matters for how this is underwritten.

Cognishine leads with the library. That is the wrong asset. A therapist with a general-purpose model can generate a picture-naming task in thirty seconds, and that gets easier every year. A large activity library is a customer acquisition asset with a declining half-life.

The barrier is the six sole-supplier notices. **A language model can produce an activity. It cannot produce a written professional opinion that a supplier is the only one able to deliver, followed by a public window in which any competitor could object.** That status has to be earned again at each new purchase, so it is a lead rather than a lock. But each renewal starts from a document the hospital has already signed once.

The content is copyable in a day. A record of six public hospitals certifying the same supplier is not.

Traction

Verifiable in a government database. Six Israeli public hospitals, eight notices in all, every one under the sole-supplier exemption.

Verifiable from the customer's side. In December 2023 Sheba Medical Center selected Cognishine as one of two start-ups, from 80 to 90

applicants, for a pilot in its rehabilitation ward for wounded soldiers. In March 2026 the company announced a strategic design partnership with Sheba's ARC Innovation Center. No financial terms were published either time. I read this as clinical validation rather than revenue.

Verifiable through a regulator. Liverpool University Hospitals NHS Foundation Trust is running a feasibility study registered with the UK Health Research Authority, REC 24/YH/0150, favourable opinion July 2024, eighty patients across intensive care and acute stroke. It lists the Israel Innovation Authority as a collaborator, was still recruiting after its estimated completion date of July 2026, and has published no results.

Published by the company, with named clinicians. Seven therapists at Barking, Havering and Redbridge NHS Trust. Linguistic Resolutions, an independent UK provider, for an intensive aphasia programme; the company says fifty therapists, the provider's own site lists about thirty. An agreement with ORITEL, the International Organization of Telethons, starting with 29 centres in Mexico, Puerto Rico and Chile, with no terms disclosed (August 2025). And one of five start-ups in EIT Health and Abbott's Start-ups Meet Pharma programme (July 2025).

Named by the company, not confirmed independently. The Clalit and Meuhedet health funds, which the CEO said had bought the service in a 2023 interview. Sderot municipality and Shalvata, named in a 2026 article. Four UK providers in the company's own case studies. Meuhedet's current pilot is funded through an Innovation Authority and Ministry of Health programme, so it is a grant rather than a sale.

How many therapists pay is not disclosed. My estimate, labelled as one, is about 200 today: the approved hospital ceilings divided by the list price, plus the therapists named in the company's UK case studies. The 5,000 therapists shown on the company's site is the sign-up count from the first month of its 2021 US launch, as the CEO described it in 2023.

Team

Nimrod Zilcha, co-founder and CEO. Economics and an MBA from Ben-Gurion, analyst at Intel managing roughly $400M in budgets, then CEO of a food export company. Non-clinical.

Meital Karni Buchman, co-founder and clinical lead. An occupational therapist with more than twenty years in the field, and the source of the clinical judgment the product rests on.

Between 13 and 31 people, depending on the source. Sagi Karni is CTO and CPO; the company site does not list him as a founder, while Startup Nation Central does. The advisory board includes Prof. Patrice Weiss, a long-

standing researcher in rehabilitation technology. **Two founders, one commercial and one clinical, with a full-time technical lead: a fit for a product that has to satisfy both a therapist and a procurement committee.**

Deal

$500,000 at seed, priced against a cap table that is already past the pre-seed stage.

By its own account in March 2026, Cognishine has raised about $2.5M of equity. PitchBook records $1.17M in May 2024 and $2M in April 2025. It also holds four Innovation Authority awards from 2022 to 2025, all in pilot tracks, and ₪385,435 in Ministry of Economy export support. That base is past pre-seed, which is what puts the company at seed rather than earlier.

Three things to confirm before any money moves. First, paying seats and annual revenue, by customer. Second, the cap table and the terms of the April 2025 round, since the $6.8M cap in the table below was set without them. Third, NHS DTAC and the HIPAA business associate agreement, which the company lists as in progress and which gate sales to NHS trusts and US health systems.

Risks

The verified revenue base is small. Across the active hospital notices the approved ceilings come to about ₪150,000 a year, and the company does not disclose how many therapists pay. Everything in the outcomes table rests on that number.

The library is a depreciating asset and the company leads with it. Generative models close this gap every year. Against that, six hospitals certified it as the only supplier able to deliver between 2024 and 2026, with full knowledge that such models exist. They are buying a clinically governed, reportable system, not content. The exposure is real for the self-serve tier and much weaker for the institutional one. The company has also started using generative AI itself: an AI Activity Creator, in beta, drafts patient-specific activities that a clinician approves before use.

Two compliance steps are incomplete by the company's own account. Its pricing page lists NHS DTAC and a HIPAA business associate agreement as in progress, and those gate sales to NHS trusts and US health systems. The other badges carry no certificate number or named auditor. Against that, six Israeli public hospitals bought it, which does not happen without a security review.

The direct competitor owns the clinical evidence. Constant Therapy has a randomised controlled trial, published in Frontiers in Neurology in 2021. Cognishine has published no outcomes yet; the Liverpool study is its first chance. Constant Therapy sells mainly direct to patients, at $29.99 a month in the US, while Cognishine is procured by institutions for their clinicians, which is consistent with hospitals certifying it as the only supplier. The gap becomes binding as the company scales into health systems that require published outcomes.

Outcomes

Outcomes

$500,000 at a $6.8M post-money cap. 7.35% at entry, roughly 4% at exit after dilution. Priced at the published $560 per therapist per year. Exit valued at 8x ARR. Probabilities are judgment. Paying therapists are not disclosed; my estimate is about 200 today.

Scenario	Prob.	What it looks like	Paying therapists	ARR	Exit	Return
Upside	15%	The institutional motion repeats in the UK and US and the platform becomes standard equipment in rehabilitation departments.	30,000	$16,800,000	$134M	**10.8x**
Base	40%	Steady department-by-department growth across Israel, the UK and private providers at roughly today's pricing.	8,000	$4,480,000	$36M	**2.9x**
Sideways	30%	Israel renews, the motion does not repeat abroad, and the company settles as a small profitable business.	2,000	$1,120,000	$9M	**0.7x**
Loss	15%	Contracts do not survive renewal at 2028, or the Israeli wins prove to be an artifact of the sole-source route.	0	$0	$0M	**0.0x**
Expected	**100%**	*Probability-weighted return on invested capital*				**3.0x**

The table is priced off the company’s list price. The Israeli contracts are too small to anchor a price, because each covers a handful of seats. The unit that matters is the paying therapist.

The base case assumes 8,000 paying therapists in six to eight years, forty times my estimate of about 200 today, which is why paying seats are the first thing to confirm. The upside case assumes 30,000, about 13% of a US professional body of roughly 230,000 members.

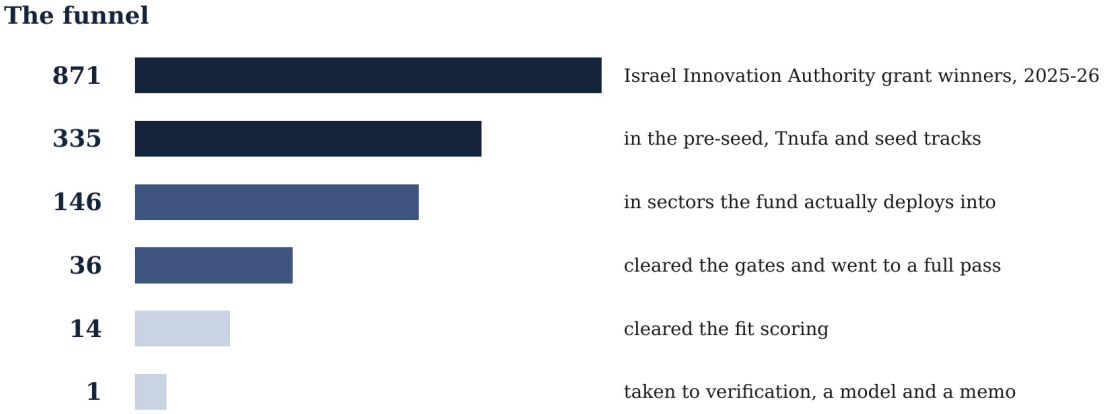
Conclusion

The only company in a screen of 146 whose customers I could find in a primary government record: six public hospitals, each of which certified it as the only supplier able to deliver. Against that, the paying base is small and undisclosed, the clinical evidence is not yet published, and two compliance steps for the NHS and US health systems are still in progress.

The public record is stronger than the visible revenue, and that is worth one meeting to test. **I recommend a first meeting, and a $500,000 seed investment if the three items under Deal check out.**

How this analysis was done

Tal Rosenberg, August 2026, revised October 2026



The screen

Three stages, ordered by cost. Cheap binary tests on everything, scored judgment second, expensive diligence only on what survives.

Gates. Binary. Any single failure removes the company.

- Two or more founders, all full-time
- A working product a customer could use
- Not a thin layer over a public model, tested as: could a competent team rebuild the core over a weekend on a public API?
- Economics that can support a venture outcome
- *Whether the founders can be argued with. Not assessable from outside, recorded as such in every file and carried to the meeting rather than dropped.*

Fit. Scored zero to three.

- Quality of validation. A named paying customer and a named pilot are deliberately not the same evidence
- Product maturity
- Founder-market fit
- Defensibility, with the mechanism named. If I could not name it in one sentence, the score was one or lower

Fundamentals. Finalists only.

- Market built from the bottom up, not from an analyst report
- Whether a budget line already exists at the buyer
- Whether price and sales motion agree
- Whether there is a route to market that does not depend on hiring a sales team first
- Whether a twelve-month story is legible from outside

Two rules throughout. Any score above the floor needed a citable source. And “I looked and it is not there” was recorded separately from “there is no way to tell from outside,” because collapsing those two is how a screen flatters whoever built it.

Sources, and why these

The pool came from a grant register rather than a startup database. Companies list themselves on databases, so the filter is willingness to fill in a form. A government committee had already assessed the technology here, and because the grant is non-dilutive that selection costs the company nothing in eligibility.

Verification came from three places, none of them the company. The government procurement register, which holds the published notices and the sole-supplier exemptions behind them. The customer’s own publication. A regulator’s registry, for the UK clinical study. All three are in the memo, and claims that rest only on the company are labelled as such.

How the outcome was priced

The unit is the paying therapist, because that is how the company prices: £37.50 per therapist per month on its Pro plan, published on its own site. A fifty-therapist department is roughly $28,000 a year at list.

The Israeli contract values are not used as the pricing anchor, because each covers only a handful of seats. **Paying therapists are not disclosed, so I estimated them:** the approved hospital ceilings divided by the list price, plus the therapists named in the company’s UK case studies, which gives about 200. It is an estimate and is labelled as one.

Exit is valued at eight times ARR, the middle of the range for vertical healthcare software with contracted institutional customers, against roughly 4% ownership at exit. The table reports whole scenarios rather than a decimal-place expected value, because the assumptions do not carry that precision.

What this analysis had access to

Public sources only: government procurement records, regulators’ registries, company registries, trade press and the company’s own materials. **No data room, no founder meeting, no customer calls.**

Those three carry most of the weight in a real decision, and in a fund the customer references would be the longest section of a memo like this. What follows from that is a limit worth stating plainly: **this cannot assess founders, which is most of what decides an outcome at this stage.** Where the accessible sources ran out I have said so rather than filled the gap.

It is a filter for which conversations are worth having, not a substitute for having them.